

IPS Problem List

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HL7 FHIR IPS IG - Problem List

The IPS problem section lists and describes clinical problems or conditions currently being monitored for the patient. If no information about problems is available or there are no known problems, this is documented in the section using emptyReason (at Composition.section) or a referenced resource (at Composition.section.entry).

- **Resource Profile Condition (IPS):** <https://build.fhir.org/HL7/fhir-ips/StructureDefinition-Condition-uv-ips.html>
- **Resource Condition:** <https://hl7.org/fhir/R4/condition.html>

openEHR CKM

- **IPS Problem List Template:** <https://ckm.openehr.org/ckm/templates/1013.26.1363>
- **Archetypes:**
 - Problem/Diagnosis: <https://ckm.openehr.org/ckm/archetypes/1013.1.169>
 - Problem/Diagnosis qualifier: <https://ckm.openehr.org/ckm/archetypes/1013.1.6653>
 - Clinical evidence: <https://ckm.openehr.org/ckm/archetypes/1013.1.1971>
 - Exclusion - global: <https://ckm.openehr.org/ckm/archetypes/1013.1.2733>
 - Absence of information: <https://ckm.openehr.org/ckm/archetypes/1013.1.4213>
 - Ad hoc heading: <https://ckm.openehr.org/ckm/archetypes/1013.1.631>

Mappings

- **Concept Map Equivalence Code System:** <https://build.fhir.org/ig/HL7/fhir-extensions/CodeSystem-concept-map-equivalence.html>
- Items marked as 'Not mapped' are technical artifacts (normally handled by openEHR Reference Model attributes) that are out of scope for the clinical mapping process.
- openEHR IPS Problem List Template → FHIR Condition (IPS) Profile Mapping
- FHIR Condition (IPS) Profile → openEHR IPS Problem List Template Mapping

openEHR IPS Problem List Template → FHIR Condition (IPS) Profile Mapping

openEHR					FHIR				Map Equivalence	Comment
Archetype	Data Element(s)	Occ.	Type	Description	Name	Card.	Type	Description & Constraints		
Ad hoc heading Renamed to 'Problem List' in the template		1..1	SECTION archetype	A generic section header which should be renamed in a template to suit a specific clinical context.	Composition.section: sectionProblems	1..1	BackboneElement	The IPS problem section lists and describes clinical problems or conditions currently being monitored for the patient.		
Exclusion - global		0..1	EVALUATION archetype	An overall statement of exclusion about all Problems/ diagnoses, Family history, Medications, Procedures, Adverse reactions or other clinical items that are either not currently present, or have not been present in the past.						
Exclusion - global	Global exclusion of problems/ diagnoses • No known problems	1..1	DV_TEXT	An overall statement of exclusion about all Problems/ diagnoses, Family history, Medications, Procedures, Adverse reactions or other clinical items. For example: 'No previous diagnoses', 'No known family history', 'No known history of adverse reactions', 'Never taken any medications' or 'No known operations or significant procedures'.	Composition.section: sectionProblems.emptyReason	0..1	CodeableConcept	nilknown notasked withheld unavailable notstarted closed If the section is empty, why the list is empty. An empty section typically has some text explaining the empty reason. The code 'nilknown' SHOULD be taken from ListEmptyReasons: If a section is empty, why it is empty. • nilknown: Clinical judgment that there are no known items for this list after reasonable investigation. Note that this a positive statement by a clinical user, and not a default position asserted by a computer system in the lack of other information. Example uses: * For diagnoses, problems and procedures: the patient or patient's agent/ guardian has asserted that there is no known event to record.	narrower	In openEHR, the reason why a problem list is empty can be handled by two separate archetypes (Exclusion - global and Absence of information), whereas in FHIR the reasons are combined in a single valueset.
Absence of information		0..1	EVALUATION archetype	Statement that specified health information is not available for inclusion in the health record or extract at the time of recording.						
Absence of information	Absence statement • No information about current problems	1..1	DV_TEXT	Positive statement that no information is available. For example: "No information available about adverse reactions"; "No information available about problems or diagnoses"; "No information available about previous procedures performed"; or "No information available about medications used".						
Absence of information	Reason for absence	0..1	DV_TEXT • Not asked • Information withheld • Unavailable • Not started • Closed	Description of the reason why there is no information available. For example: patient is unconscious or refuses to provide information. Coding the reason with a terminology is desirable, if possible.	Composition.section: sectionProblems.emptyReason	0..1	CodeableConcept	nilknown notasked withheld unavailable notstarted closed If the section is empty, why the list is empty. An empty section typically has some text explaining the empty reason. The codes SHOULD be taken from ListEmptyReasons: If a section is empty, why it is empty. • notasked: The investigation to find out whether there are items for this list has not occurred. • withheld: The content of the list was not provided due to privacy or confidentiality concerns. Note that it should not be assumed that this means that the particular information in question was withheld due to its contents - it can also be a policy decision. • unavailable: Information to populate this list cannot be obtained; e.g. unconscious patient. • notstarted: The work to populate this list has not yet begun. • closed: This list has now closed or has ceased to be relevant or useful.	narrower	In openEHR, the reason why a problem list is empty can be handled by two separate archetypes (Exclusion - global and Absence of information), whereas in FHIR the reasons are combined in a single valueset. FHIR valueset (excluding 'nilknown') added to the template.
Problem/Diagnosis		0..*	EVALUATION archetype	Details about a single identified health condition, injury, disability or any other issue which impacts on the physical, mental and/or social well-being of an individual. Clear delineation between the scope of a problem versus a diagnosis is not easy to achieve	Condition (IPS)	0..*	Condition	A clinical condition, problem, diagnosis, or other event, situation, issue, or clinical concept that has risen to a level of concern.	wider	The definitions of the concept mean the same thing, but the openEHR archetype explicitly excludes : • differential diagnoses • details about pregnancy • assessments about health risk or potential problems

				in practice. For the purposes of clinical documentation with this archetype, problem and diagnosis are regarded as a continuum, with increasing levels of detail and supportive evidence usually providing weight towards the label of 'diagnosis'.						
Problem/Diagnosis	Problem/Diagnosis name	1..1	DV_CODED_TEXT Binding: Problems - IPS	Identification of the problem or diagnosis, by name. Coding of the name of the problem or diagnosis with a terminology is preferred, where possible.	Condition.code	1..1	CodeableConcept IPS	Identification of the condition, problem or diagnosis Binding: Problems - IPS (preferred); Code for a clinical problem (or absent/unknown problem) that is selected from SNOMED CT	wider	The FHIR valueset used here includes 'all descendants or self of 160245001 [No current problems or disability (situation)]', but in openEHR this is modelled using the Exclusion - global archetype (see above).
Problem/Diagnosis	Body site	0..*	DV_TEXT Binding: SNOMEDCT BodyStructures	Identification of a simple body site for the location of the problem or diagnosis. Coding of the name of the anatomical location with a terminology is preferred, where possible. Use this data element to record precoordinated anatomical locations. If the requirements for recording the anatomical location are determined at run-time by the application or require more complex modelling such as relative locations then use the CLUSTER.anatomical_location or CLUSTER.relative_location within the 'Structured anatomical location' SLOT in this archetype. Occurrences for this data element are unbounded to allow for clinical scenarios such as describing a rash in multiple locations but where all of the other attributes are identical. If the anatomical location is included in the Problem/ diagnosis name via precoordinated codes, this data element becomes redundant.	Condition.bodySite	0..*	CodeableConcept	Anatomical location, if relevant Binding: SNOMEDCTBodyStructures (preferred)	equal	
Problem/Diagnosis	Date/time of onset	0..1	DV_DATE_TIME	Estimated or actual date/time that signs or symptoms of the problem/diagnosis were first observed. Data captured/imported as "Age at onset" should be converted to a date using the subject's date of birth.	Condition.onset[x]:onsetDateTime	0..1	dateTime	Estimated or actual date or date-time the condition began, in the opinion of the clinician.	wider	In FHIR, onset is a choice of dateTime , Age , Period , Range or string .
Problem/Diagnosis	Severity Mild: The problem or diagnosis does not interfere with normal activity or may cause damage to health if left untreated. Moderate: The problem or diagnosis causes interference with normal activity or will damage health if left untreated. Severe: The problem or diagnosis prevents normal activity or will seriously damage health if left untreated.	0..1	DV_CODED_TEXT / DV_TEXT	An assessment of the overall severity of the problem or diagnosis. If severity is included in the Problem/diagnosis name via precoordinated codes, this data element becomes redundant. Note: more specific grading of severity can be recorded using the Specific details SLOT.	Condition.severity	0..1	CodeableConcept IPS	Subjective severity of condition Binding: Condition/ DiagnosisSeverity (preferred) 24484000 Severe 6736007 Moderate 255604002 Mild	equal	
Problem/Diagnosis	Specific details → Clinical evidence	0..*	Slot	Details that are additionally required to record as unique attributes of this problem or diagnosis.						
Clinical evidence Renamed to 'Stage' in the template		0..*	CLUSTER archetype	Details about findings that support a clinical assertion.	Condition.stage	0..*	BackboneElement	Stage/grade, usually assessed formally	equivalent	
Clinical evidence	Evidence	0..1	DV_TEXT	Identification of an item of clinical evidence by name or type, either as a single result or as a grouping of results. It is recommended that 'Evidence' should be coded with a terminology, where possible. For example: 'Mantoux test', 'AFP level', 'tumour volume', 'genetic tests', 'BRCA1 test', 'histopathology result', 'physical examination finding', 'intraoperative finding', 'FIGO stage', 'symptom', 'tumour grading', 'clinical impression'. For use cases where the evidence is sufficiently identified in 'Result' or 'Structured result', this data element may be redundant and has therefore been made optional.	Condition.stage.type	0..1	CodeableConcept	Kind of staging Binding: ConditionStageType (example); Codes describing the kind of condition staging (e.g. clinical or pathological).	equivalent	
Clinical evidence	Result	0..1	DV_CODED_TEXT / DV_TEXT	The result or finding that supports the assertion. For example: '11 mm', '13 µg/L', '2.5 ml' or 'BRCA1 positive'. 'Result' can be coded with a terminology, where appropriate.	Condition.stage.summary	0..1	CodeableConcept	Simple summary (disease specific) Binding: ConditionStage (example); Codes describing condition stages (e.g. Cancer stages).	equivalent	
Clinical evidence	Structured result	0..*	SLOT	Structured details about the result.	Condition.stage.assessment	0..*	Reference (ClinicalImpression DiagnosticReport Observation)	Formal record of assessment	equivalent	
Clinical evidence		0..*	CLUSTER archetype	Details about findings that support a clinical assertion.	Condition.evidence	0..*	BackboneElement	Supporting evidence	equal	
	Result	0..*	DV_CODED_TEXT / DV_TEXT	The result or finding that supports the assertion. For example: '11 mm', '13 µg/L', '2.5 ml' or 'BRCA1 positive'. 'Result' can be coded with a terminology, where appropriate.	Condition.evidence.code	0..*	CodeableConcept	Manifestation/symptom Binding: ManifestationAndSymptomCodes (example); Codes that describe the manifestation or symptoms of a condition.	equivalent	
	Structured result	0..*	SLOT	Structured details about the result.	Condition.evidence.detail	0..*	Reference (Resource)	Supporting information found elsewhere	equivalent	
Problem/Diagnosis	Date/time of resolution	0..1	DV_DATE_TIME	Estimated or actual date/time of resolution or remission for this problem or diagnosis, as determined by a healthcare professional. Partial dates are acceptable. If the subject of care is under the age of one year, then the	Condition.abatement[x]:abatementDateTime	0..1	dateTime	The date or estimated date that the condition resolved or went into remission. This is called "abatement" because of the many overloaded connotations associated with "remission" or "resolution" - Conditions are never really resolved, but they can abate.	wider	In FHIR, abatement is a choice of dateTime , Age , Period , Range or string .

				complete date or a minimum of the month and year is necessary to enable accurate age calculations - for example, if used to drive decision support. Data captured/imported as "Age at time of resolution" should be converted to a date using the subject's date of birth.						
Problem/Diagnosis	Status → <i>Problem/Diagnosis qualifier</i>	0..*	Slot	Structured details for location-, domain-, episode- or workflow-specific aspects of the diagnostic process.						
<i>Problem/Diagnosis qualifier</i>		0..1	CLUSTER archetype	Contextual or temporal qualifier for a specified problem or diagnosis.						
Problem/Diagnosis qualifier	Active/Inactive? Active: The problem or diagnosis is currently active and clinically relevant. Inactive: The problem or diagnosis is not completely resolved but is inactive or felt less relevant to the current clinical context.	0..1	DV_CODED_TEXT	Category that supports division of problems and diagnoses into Active or Inactive problem lists. The Active/Inactive and Current/ Past data elements have similar clinical impact but represent slightly different semantics. Both are actively used in different clinical settings, but usually not together. If a Current/Past qualifier is recorded, then this data element is likely to be redundant. An exception where a condition can be current but inactive is asthma that is not causing acute symptoms.	Condition. <i>clinicalStatus</i>	0..1	CodeableConceptIPS	active recurrence relapse inactive remission resolved Binding: <i>ConditionClinicalStatusCodes</i> (required): The clinical status of the condition or diagnosis. active: The subject is currently experiencing the symptoms of the condition or there is evidence of the condition. inactive: The subject is no longer experiencing the symptoms of the condition or there is no longer evidence of the condition.	inexact	Clinical status incompletely captures aspects of multiple qualifier axes related to a diagnosis: - active/inactive - occurrence - first occurrence/ recurrence - remission status - in remission/ not in remission - resolution phase - Not resolving [Problem or diagnosis is not progressing satisfactorily through the normal stages of restoration or healing towards resolution.] - Resolving [Problem or diagnosis is progressing satisfactorily through the normal stages of restoration or healing and can be considered resolved.] - Relapsed [Problem or diagnosis has deteriorated after a period of temporary improvement.] - Indeterminate [It is not possible to determine the resolution or healing status of the problem or diagnosis.] Nictiz (NL) has done some work on the mapping and proposed what should change in FHIR to make these more mappable (see attachment). <i>Condition - clinicalStatus valueset - Engels.pdf</i>
Problem/Diagnosis qualifier	Resolution phase Resolved: Problem or diagnosis has completed the normal phases of restoration or healing and can be considered resolved. Relapsed: Problem or diagnosis has deteriorated after a period of temporary improvement.	0..1	DV_CODED_TEXT	Phase of healing for an acute problem or diagnosis. For example: tracking the progress of resolution of a middle ear infection.	Condition. <i>clinicalStatus</i>	0..1	CodeableConceptIPS	active recurrence relapse inactive remission resolved Binding: <i>ConditionClinicalStatusCodes</i> (required): The clinical status of the condition or diagnosis. relapse: The subject is experiencing a return of a condition, or signs and symptoms after a period of improvement or remission, e.g. relapse of cancer, multiple sclerosis, rheumatoid arthritis, systemic lupus erythematosus, bipolar disorder, [psychotic relapse of] schizophrenia, etc. resolved: The subject is no longer experiencing the symptoms of the condition and there is a negligible perceived risk of the symptoms returning.	inexact	
Problem/Diagnosis qualifier	Remission status In remission: No ongoing signs or symptoms of the disease have been identified.	0..1	DV_CODED_TEXT	Status of the remission of an incurable diagnosis. For example: the status of a cancer or haematological diagnosis.	Condition. <i>clinicalStatus</i>	0..1	CodeableConceptIPS	active recurrence relapse inactive remission resolved Binding: <i>ConditionClinicalStatusCodes</i> (required): The clinical status of the condition or diagnosis. remission: The subject is no longer experiencing the symptoms of the condition, but there is a risk of the symptoms returning.	inexact	
Problem/Diagnosis qualifier	Occurrence Recurrence: New occurrence of the same problem or diagnosis after a previous episode was resolved.	0..1	DV_CODED_TEXT	Category of the occurrence for this problem or diagnosis. This data element can be an additional qualifier to the 'New' value in the 'Episodicity' value set, that is a condition such as asthma can have recurring new episodes that have periods of resolution in between. However it can be important to identify the first ever episode of asthma from all of the other episodes.	Condition. <i>clinicalStatus</i>	0..1	CodeableConceptIPS	active recurrence relapse inactive remission resolved Binding: <i>ConditionClinicalStatusCodes</i> (required): The clinical status of the condition or diagnosis. recurrence: The subject is experiencing a re-occurrence or repeating of a previously resolved condition, e.g. urinary tract infection, pancreatitis, cholangitis, conjunctivitis.	inexact	
Problem/Diagnosis	Diagnostic certainty Suspected: The diagnosis has been identified with a low level of certainty. Probable: The diagnosis has been identified with a high level of certainty. Confirmed: The diagnosis has been confirmed against recognised criteria.	0..1	DV_CODED_TEXT / DV_TEXT	The level of confidence in the identification of the diagnosis. If an alternative valueset is required, these values can be added to the DV_TEXT data type in a template.	Condition. <i>verificationStatus</i>	0..1	CodeableConcept	unconfirmed provisional differential confirmed refuted entered-in-error Binding: <i>ConditionVerificationStatus</i> (required): The verification status to support or decline the clinical status of the condition or diagnosis. confirmed: There is sufficient diagnostic and/or clinical evidence to treat this as a confirmed condition.	inexact	Verification status incompletely captures aspects of multiple qualifier axes related to a diagnosis: - confirmation - exclusion (refuted) - diagnostic process (provisional) - clinical decision-making (differential - one of a list of candidate diagnoses generated during clinical reasoning and used to guide investigation and exclusion) Only 'Confirmed' can be safely mapped between the openEHR and FHIR valuesets at this point in time. There is no clear way of mapping 'Suspected' and 'Probable' to the FHIR values as their meaning can be interpreted and handled differently in different contexts and locations.

										It is also unclear what the use of 'refuted' means in the IPS context and how it should be handled. Differential diagnosis is covered by a different openEHR archetype - Differential diagnoses, which has not been included in the template as it is recommended that differential diagnoses should not be part of the IPS. 'Entered-in-error' is a record-management state and should result in deletion or retraction in an EHR, not the assignment of condition verification status. For more information, see this openEHR Discourse thread: https://discourse.openehr.org/t/degree-of-certainty-vs-status-in-problem-diagnosis/11561
	Comment	0..1	DV_TEXT	Additional narrative about the problem or diagnosis not captured in other fields.	Condition.note	0..*	Annotation	Additional information about the Condition	wider	Semantic intent is equal but the occurrences differ.

FHIR Condition (IPS) Profile → openEHR IPS Problem List Template Mapping

FHIR				openEHR					Map Equivalence	Comment
Name	Card.	Type	Description & Constraints	Archetype	Data Element(s)	Occ.	Type	Description		
Condition (IPS)	0..*	Condition	A clinical condition, problem, diagnosis, or other event, situation, issue, or clinical concept that has risen to a level of concern.	Problem/Diagnosis		0..*	EVALUATION archetype	Details about a single identified health condition, injury, disability or any other issue which impacts on the physical, mental and/or social well-being of an individual. Clear delineation between the scope of a problem versus a diagnosis is not easy to achieve in practice. For the purposes of clinical documentation with this archetype, problem and diagnosis are regarded as a continuum, with increasing levels of detail and supportive evidence usually providing weight towards the label of 'diagnosis'.	narrower	The definitions of the concept mean the same thing, but the openEHR archetype explicitly excludes: - differential diagnoses - details about pregnancy - assessments about health risk or potential problems
Condition.id	0..1	id	Logical id of this artifact						Not mapped	
Condition.meta	0..1	Meta	Metadata about the resource						Not mapped	
Condition.implicitRules	0..1	uri	A set of rules under which this content was created						Not mapped	
Condition.language	0..1	code	Language of the resource content Binding: CommonLanguages (preferred): A human language.						Not mapped	
Condition.text	0..1	Narrative	Text summary of the resource, for human interpretation						Not mapped	
Condition.contained	0..*	Resource	Contained, inline Resources						Not mapped	
Condition.extension	0..*	Extension	Additional content defined by implementations						Not mapped	
Condition.modifierExtension	0..*	Extension	Extensions that cannot be ignored						Not mapped	
Condition.identifier	0..*	Identifier	External ids for this condition						Not mapped	
Condition.clinicalStatus	0..1	CodeableConceptIPS	active recurrence relapse inactive remission resolved Binding: ConditionClinicalStatusCodes (required): The clinical status of the condition or diagnosis.	Problem/Diagnosis qualifier	Active/Inactive? Active: The problem or diagnosis is currently active and clinically relevant. Inactive: The problem or diagnosis is not completely resolved but is inactive or felt less relevant to the current clinical context. Resolution phase Resolved: Problem or diagnosis has completed the normal phases of restoration or healing and can be considered resolved. Relapsed: Problem or diagnosis has deteriorated after a period of temporary improvement. Remission status In remission: No ongoing signs or symptoms of the disease have been identified. Occurrence Recurrence: New occurrence of the same problem or diagnosis after a previous episode was resolved.		inexact	Clinical status incompletely captures aspects of multiple qualifier axes related to a diagnosis: - active/inactive - occurrence - first occurrence/ recurrence - remission status - in remission/ not in remission - resolution phase - Not resolving [Problem or diagnosis is not progressing satisfactorily through the normal stages of restoration or healing towards resolution.] - Resolving [Problem or diagnosis is progressing satisfactorily through the normal stages of restoration or healing towards resolution.] - Resolved [Problem or diagnosis has completed the normal phases of restoration or healing and can be considered resolved.] - Relapsed [Problem or diagnosis has deteriorated after a period of temporary improvement.] - Indeterminate [It is not possible to determine the resolution or healing status of the problem or diagnosis.] Nicitz (NL) has done some work on the mapping and proposed what should change in FHIR to make these more mappable (see attachment). Condition - clinicalStatus valueset - Engels.pdf		
Condition.verificationStatus	0..1	CodeableConcept	unconfirmed provisional differential confirmed refuted entered-in-error Binding: ConditionVerificationStatus (required): The verification status to support or decline the clinical status of the condition or diagnosis.	Problem/Diagnosis	Diagnostic certainty Suspected: The diagnosis has been identified with a low level of certainty. Probable: The diagnosis has been identified with a high level of certainty. Confirmed: The diagnosis has been confirmed against recognised criteria.	0..1	DV_CODED_TEXT / DV_TEXT	The level of confidence in the identification of the diagnosis. If an alternative valueset is required, these values can be added to the DV_TEXT data type in a template.	inexact	Verification status incompletely captures aspects of multiple qualifier axes related to a diagnosis: - confirmation - exclusion (refuted) - diagnostic process (provisional) - clinical decision-making (differential - one of a list of candidate diagnoses generated during clinical reasoning and used to guide investigation and exclusion) Only 'confirmed' can be safely mapped between the FHIR and openEHR valuesets at this point in time.

										Equivalent to 'unconfirmed' is missing and there is no clear way of mapping 'provisional' to the openEHR values. It is also unclear what the use of 'refuted' means in the IPS context and how it should be handled. Differential diagnosis is covered by a different openEHR archetype - Differential diagnoses, which has not been included in the template as it is recommended that differential diagnoses should not be part of the IPS. 'Entered-in-error' is a record-management state and should result in deletion or retraction in an EHR, not the assignment of condition verification status. For more information, see this openEHR Discourse thread: https://discourse.openehr.org/t/degree-of-certainty-vs-status-in-problem-diagnosis/11561
Condition.category	0..*	CodeableConceptIPS	problem-list-item encounter-diagnosis A category assigned to the condition. In this profile, a health problem of the patient, therefore a 'problem-list-item'. Binding: Problem Type - IPS (extensible)	Problem list / Encounter					unmatched	
Condition.severity	0..1	CodeableConceptIPS	Subjective severity of condition A subjective assessment of the severity of the condition as evaluated by the clinician. Binding: Condition/ DiagnosisSeverity (preferred) 24484000 Severe 6736007 Moderate 255604002 Mild	Problem/Diagnosis	Severity Mild: The problem or diagnosis does not interfere with normal activity or may cause damage to health if left untreated. Moderate: The problem or diagnosis causes interference with normal activity or will damage health if left untreated. Severe: The problem or diagnosis prevents normal activity or will seriously damage health if left untreated.	0..1	DV_CODED_TEXT / DV_TEXT	An assessment of the overall severity of the problem or diagnosis. If severity is included in the Problem/diagnosis name via precoordinated codes, this data element becomes redundant. Note: more specific grading of severity can be recorded using the Specific details SLOT.	equal	
Condition.code	1..1	CodeableConceptIPS	Identification of the condition, problem or diagnosis Binding: Problems - IPS (preferred); Code for a clinical problem (or absent/unknown problem) that is selected from SNOMED CT	Problem/Diagnosis	Problem/Diagnosis name Binding: Problems - IPS	1..1	DV_CODED_TEXT	Identification of the problem or diagnosis, by name. Coding of the name of the problem or diagnosis with a terminology is preferred, where possible.	equal	
Condition.bodySite	0..*	CodeableConcept	The anatomical location where this condition manifests itself. Binding: SNOMEDCTBodyStructures (preferred)	Problem/Diagnosis	Body site Binding: SNOMEDCTBodyStructures	0..*	DV_TEXT	Identification of a simple body site for the location of the problem or diagnosis. Coding of the name of the anatomical location with a terminology is preferred, where possible. Use this data element to record precoordinated anatomical locations. If the requirements for recording the anatomical location are determined at run-time by the application or require more complex modelling such as relative locations then use the CLUSTER.anatomical_location or CLUSTER.relative_location within the 'Structured anatomical location' SLOT in this archetype. Occurrences for this data element are unbounded to allow for clinical scenarios such as describing a rash in multiple locations but where all of the other attributes are identical. If the anatomical location is included in the Problem/ diagnosis name via precoordinated codes, this data element becomes redundant.	equal	
Condition.subject	1..1	Reference(Patient (IPS))	Who has the condition?						Not mapped	
Condition.subject.id	0..1	string	Unique id for inter-element referencing						Not mapped	
Condition.subject.extension	0..*	Extension	Additional content defined by implementations						Not mapped	
Condition.subject.reference	1..1	string	Literal reference, Relative, internal or absolute URL						Not mapped	
Condition.subject.type	0..1	uri	Type the reference refers to (e.g. "Patient") Binding: ResourceType (extensible); A resource (or, for logical models, the URT of the logical model).						Not mapped	
Condition.subject.identifier	0..1	Identifier	Logical reference, when literal reference is not known						Not mapped	
Condition.subject.display	0..1	string	Text alternative for the resource						Not mapped	
Condition.encounter	0..1	Reference(Encounter)	Encounter created as part of						Not mapped	
Condition.onset[x]	0..1		Estimated or actual date or date-time the condition began, in the opinion of the clinician. Age is generally used when the patient reports an age at which the Condition began to occur.						narrower	In openEHR, Date/time of onset can only be DV_DATE_TIME.
Condition.onset[x].onsetDateTime		dateTime		Problem/Diagnosis	Date/time of onset	0..1	DV_DATE_TIME	Estimated or actual date/time that signs or symptoms of the problem/diagnosis were first observed. Data captured/imported as "Age at onset" should be converted to a date using the subject's date of birth.	equal	
Condition.onset[x].onsetAge		Age							unmatched	

Condition.onset[x]: onsetPeriod		Period							unmatched	
Condition.onset[x]: onsetRange		Range							unmatched	
Condition.onset[x]: onsetString		string							unmatched	
Condition.abatement[x]	0..1		The date or estimated date that the condition resolved or went into remission. This is called "abatement" because of the many overloaded connotations associated with "remission" or "resolution" - Conditions are never really resolved, but they can abate.						narrower	In openEHR, Date/time of resolution can only be DV_DATE_TIME.
Condition.abatement[x]: abatementDateTime		dateTime		Problem/Diagnosis	Date/time of resolution	0..1	DV_DATE_TIME	Estimated or actual date/time of resolution or remission for this problem or diagnosis, as determined by a healthcare professional. Partial dates are acceptable. If the subject of care is under the age of one year, then the complete date or a minimum of the month and year is necessary to enable accurate age calculations - for example, if used to drive decision support, Date captured/imported as "Age at time of resolution" should be converted to a date using the subject's date of birth.	equal	
Condition.abatement[x]: abatementAge		Age							unmatched	
Condition.abatement[x]: abatementPeriod		Period							unmatched	
Condition.abatement[x]: abatementRange		Range							unmatched	
Condition.abatement[x]: abatementString		string							unmatched	
Condition.recordedDate	0..1	dateTime	Date record was first recorded						Not mapped	
Condition.recorder	0..1	Reference (Practitioner PractitionerRole Patient RelatedPerson)	Who recorded the condition						Not mapped	
Condition.asserter	0..1	Reference (Practitioner PractitionerRole Patient RelatedPerson)	Person who asserts this condition						Not mapped	
Condition.stage	0..*	BackboneElement	Clinical stage or grade of a condition. May include formal severity assessments.	Clinical evidence Renamed to 'Stage' in the template		0..*	CLUSTER archetype	Details about findings that support a clinical assertion.	equivalent	
Condition.stage.id	0..1	string	Unique id for inter-element referencing						Not mapped	
Condition.stage.extension	0..*	Extension	Additional content defined by implementations						Not mapped	
Condition.stage.modifierExtension	0..*	Extension	Extensions that cannot be ignored even if unrecognized						Not mapped	
Condition.stage.summary	0..1	CodeableConcept	A simple summary of the stage such as "Stage 3". The determination of the stage is disease-specific. Binding: ConditionStage (example); Codes describing condition stages (e.g. Cancer stages).	Clinical evidence Renamed to 'Stage' in the template	Result	0..1	DV_CODED_TEXT / DV_TEXT	The result or finding that supports the assertion. For example: '11 mm', '13 µg/L', '2.5 ml' or 'BRCA1 positive'. 'Result' can be coded with a terminology, where appropriate.	equivalent	
Condition.stage.assessment	0..*	Reference (ClinicalImpression DiagnosticReport Observation)	Reference to a formal record of the evidence on which the staging assessment is based.	Clinical evidence Renamed to 'Stage' in the template	Structured result	0..*	SLOT	Structured details about the result.	equivalent	
Condition.stage.type	0..1	CodeableConcept	The kind of staging, such as pathological or clinical staging. Binding: ConditionStageType (example); Codes describing the kind of condition staging (e.g. clinical or pathological).	Clinical evidence Renamed to 'Stage' in the template	Evidence	0..1	DV_CODED_TEXT / DV_TEXT	Identification of an item of clinical evidence by name or type, either as a single result or as a grouping of results. It is recommended that 'Evidence' should be coded with a terminology, where possible. For example: 'Mantoux test', 'AFP level', 'tumour volume', 'genetic tests', 'BRCA1 test', 'histopathology result', 'physical examination finding', 'intraoperative finding', 'FIGO stage', 'symptom', 'tumour grading', 'clinical impression'. For use cases where the evidence is sufficiently identified in 'Result' or 'Structured result', this data element may be redundant and has therefore been made optional.	equivalent	
Condition.evidence	0..*	BackboneElement	Supporting evidence / manifestations that are the basis of the Condition's verification status, such as evidence that confirmed or refuted the condition.	Clinical evidence		0..*	CLUSTER archetype	Details about findings that support a clinical assertion.	equal	
Condition.evidence.id	0..1	string	Unique id for inter-element referencing						Not mapped	
Condition.evidence.extension	0..*	Extension	Additional content defined by implementations						Not mapped	
Condition.evidence.modifierExtension	0..*	Extension	Extensions that cannot be ignored even if unrecognized						Not mapped	
Condition.evidence.code	0..*	CodeableConcept	A manifestation or symptom that led to the recording of this condition. Binding: ManifestationAndSymptomCodes (example); Codes that describe the manifestation or symptoms of a condition.	Clinical evidence	Result	0..*	DV_CODED_TEXT / DV_TEXT	The result or finding that supports the assertion. For example: '11 mm', '13 µg/L', '2.5 ml' or 'BRCA1 positive'. 'Result' can be coded with a terminology, where appropriate.	equivalent	
Condition.evidence.detail	0..*	Reference(Resource)	Links to other relevant information, including pathology reports.	Clinical evidence	Structured result	0..*	SLOT	Structured details about the result.	equivalent	

Condition.note	0..*	Annotation	Additional information about the Condition. This is a general notes/ comments entry for description of the Condition, its diagnosis and prognosis.	Problem/Diagnosis	Comment	0..1	DV_TEXT	Additional narrative about the problem or diagnosis not captured in other fields.	equal	Semantic intent is equal but the occurrences differ.
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No labels

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